6 Voluntary and Involuntary Psychiatric Hospitalization

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Clinical Vignette

You are working as an attending psychiatrist in a psychiatric emergency department to which a 22-year-old man is brought in by his parents. According to the parents, the patient, who is attending his third year of college in a different state, suddenly returned home 2 weeks ago and announced that he planned to drop out of school. The parents are concerned as their son spends most of his time in his bedroom, refuses to bathe, and is increasingly preoccupied with concerns that the government is spying on him through the TV and his cell phone. He has taken multiple steps to protect himself, including destroying his cell phone, setting his computer on fire, and taping shut all the windows. After his parents expressed concern and suggested bringing him to the hospital, the patient accused parents of conspiring with the government. He then physically assaulted his father causing him a wrist fracture. The patient eventually agreed to accompany his parents to the hospital so that his father could have his wrist evaluated, and once in the emergency room, he agreed to speak to a psychiatrist. When you meet with the patient, he agrees with the history provided by his parents but does not seem to appreciate that there is anything concerning about his behavior. He states that his father's injury was an accident but threatens further injury to parents if they insist on his psychiatric treatment. He also reports hearing voices telling him to stop talking with you, refuses treatment, and states that he wishes to return home with his parents.

What do you do? Does the patient need to be hospitalized or not? If so, how can you accomplish this over his objection and what legal justification do you have for

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making this decision? If the patient agrees to hospitalization, should he be admitted as a voluntary patient?

History of the Real Legal Case

Addington v. Texas (1979) was a landmark decision of the US Supreme Court that determined the legal certainty required for civil commitment. The Court ruled that a "clear and convincing" standard of proof is required by the Fourteenth Amendment in civil commitment proceedings [1].

Mr. Addington, who had a history of schizophrenia, violence, property damage, and threatening his family and others, was committed multiple times to Texas state mental hospitals between 1969 and 1975. In 1975, his mother filed a petition for his indefinite commitment. After her request was granted Mr. Addington requested a trial by jury to challenge his commitment. The trial court judge tasked the jury with determining if Mr. Addington was mentally ill and dangerous based on "clear, unequivocal, and convincing evidence." The jury found that Mr. Addington met the criterion, and he was indefinitely committed to the state hospital. Mr. Addington appealed to the Texas Court of Civil Appeals, conceding that he suffered from mental illness but contesting the legal standard used in his case. He argued that any standard of proof for commitment less than that of "beyond reasonable doubt" (the highest possible legal standard used in criminal court cases) violated his procedural due process rights. The Texas Court of Civil Appeals agreed with Mr. Addington and reversed the trial court's judgment. The Texas Supreme Court then reversed the appeals court decision holding that the legal standard for civil commitment is "preponderance of the evidence." Mr. Addington appealed to the US Supreme Court.

The US Supreme Court held that "preponderance of the evidence" standard was insufficient given the significant deprivation of liberty that required due process protection. On the other hand, "beyond a reasonable doubt" standard was deemed inappropriate because of the uncertainties of psychiatric diagnoses, imposing a burden that states could not meet, and erecting an unreasonable barrier to needed medical treatment. The court held that the state needs to prove the presence of an individual's mental illness and dangerousness by the standard of "clear and convincing evidence" [1].

Introduction

Although most patients with mental illness can manage their symptoms on an outpatient basis, few would question that individuals who are suicidally depressed, manically reckless, failing to thrive due to a cognitive impairment or eating disorder, or overwhelmed with persecutory delusions and aggressive urges toward others, are in need of a higher level of psychiatric care. In such cases, psychiatric hospitalization may be necessary. The majority of patients are admitted to psychiatric hospitals on a voluntary basis. However, severe mental illness may impair a person's

perception, thinking, and behavior, and compromise insight and judgment, making refusal of care relatively common in psychiatry. To ensure safety and administer necessary treatments, psychiatrists may need to resort to involuntary or civil commitment, a legal process through which an individual with symptoms of severe mental illness is court-ordered into psychiatric care.

Voluntary Hospitalization

The first US statute allowing voluntary admission was enacted in Massachusetts in 1881, but it took almost one hundred years before voluntary admissions became the norm in this country. Presently, all states permit patients to be voluntarily admitted for psychiatric hospitalization. For adolescents, the age of consent to psychiatric hospitalization ranges from 12 to 18 years [2].

Many state statutes have provisions to allow for two types of voluntary admissions. The first is under a "pure" or "informal" status where the patient can be voluntarily admitted, but must be discharged immediately upon their request, unless there are grounds to apply for involuntary commitment. A second provision occurs under "conditional" or "formal" status, where the patient agrees at the time of admission to the facility that the facility has the right to detain the patient for a brief period, usually several days, after patient requests discharge. This period gives the patient's treatment team the time to determine if the patient can be safely discharged. If a determination is made that the patient's discharge cannot be safely arranged, the treatment team may initiate for involuntary commitment proceedings. This statutorily defined waiting period also gives patients time to fully consider their decision to terminate hospitalization [2–4].

Confusion sometimes exists as to whether patients who request voluntary admission should be permitted to sign in voluntarily if there are questions about their decision-making capacity. Early statutes authorizing voluntary admissions made it an explicit requirement that a patient be competent to sign in voluntarily. More recent laws, designed to encourage patient autonomy, omit such requirements in all but a handful of states [2]. Allowing a patient to sign into the hospital voluntarily when they lack the requisite knowledge of what the hospitalization entails leaves psychiatrists and hospitals legally vulnerable. On the other hand, the procedural burden and involuntary nature of commitment, may deter patients from seeking necessary treatment.

The threshold for decision-making capacity to be voluntarily admitted to a psychiatric hospital was addressed in the 1990 US Supreme Court case of Zinermon v. Burch [2, 5]. In December 1981, Mr. Darell Burch was found on a highway in Florida, disoriented and psychotic. A passerby brought him to a private hospital where he was psychiatrically hospitalized and from which he was later transferred to Florida State Hospital. At both facilities, Mr. Burch was permitted to sign in as a voluntary patient. Mr. Burch remained hospitalized for a total of 5 months and later filed a lawsuit in federal district court against both the private and state facilities. In the lawsuit, he alleged that he was inappropriately detained in violation of his

constitutional rights when he was allowed to sign in as a voluntary patient despite lacking the capacity to do so. Mr. Burch argued that he was entitled to the procedural safeguards of the involuntary commitment procedures. The case was appealed up to the US Supreme Court, which held that in those states that require a patient to be competent before signing in voluntarily, the failure to assess competency violates patients' constitutional rights [2, 5, 6].

The question remains as to the degree of knowledge and understanding necessary to determine whether a patient has the capacity to be voluntarily admitted to a psychiatric hospital. Even patients with severe mental illness may be competent to make certain medical decisions, including admission to the psychiatric hospital. This question was addressed by the American Psychiatric Association Task Force in 1993 [2, 7, 8]. The task force established criteria for psychiatrists to apply when assessing this specific decision-making capacity. To preserve the preference of voluntary admissions, "the task force suggested that only a minimal level of capacity be required." It recommended in-hospital administrative decisions (as opposed to judicial hearings) predicated on easy-to-meet substantive standards that the patient demonstrates the ability to communicate choices and the ability to understand relevant information. These tests would be satisfied when the patient expresses "an agreement with admission and treatment in any way, verbal, behavioral or written," and display some minimal understanding of where the patient is and why [2, 7].

Involuntary Hospitalization

Involuntary commitment decisions present a unique challenge for psychiatrists [2–4, 6]. Psychiatric interventions are guided by ethical principles of medical practice—non-maleficence, autonomy, beneficence, and justice [2, 3, 9–14]. Physicians are expected to refrain from causing harm to their patients (non-maleficence) and obliged to provide treatments that are in their patients' best interest (beneficence). The principle of autonomy recognizes the right of an individual to self-determination and requires that a physician respects the authority of competent individuals to make decisions about their own medical care. The principle of justice dictates that medical benefits should be dispensed fairly. Challenges arise when principles of autonomy and beneficence are in conflict, such as when considering involuntary commitment. As clinical decisions related to depriving patients of their civil liberties carry tremendous responsibility, psychiatrists rely on laws and hospital regulations to guide their decision-making process [9–13].

Involuntary commitment deprives individuals of their freedom and other liberty interests, and as such, several procedural and substantive safeguards have been established by federal and state laws to ensure that such measures are only enacted when an individual's life is in peril [15–20]. Procedural and substantive safeguards are delineated in the Fifth and Fourteenth Amendments to the US Constitution in the due process clauses. Due process guarantees fair treatment throughout the judicial process and protects individuals from governmental abuse of power. There are two basic types of due process—procedural and substantive. Procedural due process

ensures formal proceedings that are carried out in accordance with established rules and principles. Substantive due process is a judicial requirement that enacted laws may not result in the unfair, arbitrary, or unreasonable treatment of an individual [14, 15].

Procedural due process in civil commitment includes requirement that the patient is examined by one or more psychiatrists or other qualified individuals designated by the court, provision of notice of hearing, court appointment of a legal representative, and an opportunity to participate in an adversarial hearing. At the hearing, there is an opportunity for cross-examination of witnesses and examination of evidence. The hearing may take place, depending on the state, in a district, superior, family or probate court, or at the hospital. Most civil commitment cases are heard by judges, and the decision whether to commit the individual is based on written reports and oral testimonies. A small number of states guarantee the patient the right to have a jury-rendered decision on the question of commitment; the vast majority of cases are heard solely by judges. In some states, the duration of the commitment is explicitly limited and recommitment after that specified period requires additional hearings, while other states do not specify the duration. In most states a periodic review of the patient's status is statutorily required to determine if the criteria for involuntary commitment continue to be met or if the patient should be discharged [18–21].

Substantive due process in civil commitment requires the establishment of legal standards for imposing commitment. Although jurisdictions vary regarding the details of their statutory provisions, the essential requirements in most states are that the decision-maker establishes the presence of a mental illness, that there is clear and convincing evidence that the patient presents a risk of harm to themselves (including by reason of grave disability) or others due to their mental illness, and that no less restrictive treatment setting is appropriate or available [18–21].

Statutes often define a "person with psychiatric disabilities" as anyone who has a mental or emotional condition that substantially and adversely affects his or her ability to function and who requires care and treatment. The term "dangerous to himself or others" conveys that there is a substantial risk that the individual will inflict physical harm to themselves or others. The term "gravely disabled" refers to a person who, due to mental or emotional impairment, is in danger of serious harm because of inability to secure basic needs such as food, clothing, shelter, or safety [2].

In the US legal system, there are three standards of proof in criminal or civil proceedings [15, 16, 18, 20]. The standards serve to allocate the risk of error between the parties. The lowest standard of proof is "preponderance of the evidence." This standard is met if the proposition is assessed by the fact finder to be more likely true than not. This standard of proof is used in most civil cases. At the other end of the spectrum is the highest standard of proof, "beyond a reasonable doubt," used in criminal cases where erroneous decisions may have severe consequences for the accused, such as long-term loss of liberty or even life. In such cases, the risk is imposed almost entirely on the state to prove the accused's guilt "beyond a reasonable doubt." The third and intermediate standard, "clear and convincing evidence," is used in civil commitment proceedings and requires that the trier of fact must be

firmly convinced that the evidence presented is substantially more likely to be true than not. This standard for civil commitment proceedings was established in the landmark US Supreme Court case Addington v. Texas [1].

Brief History of Involuntary Commitment Laws in the United States

Contemporary involuntary commitment statutes are the product of over two centuries of work aimed at balancing individual liberty rights and concerns for patients' best interests [2, 6]. Traditionally, the state's power to involuntarily commit individuals is derived from two basic legal doctrines: parens patriae and police power. *Parens patriae* translates from Latin as "parent of the nation." It originally referred to the sovereign's duty to care for the members of the society who were unable to care for themselves. In modern times, the term refers to a doctrine that grants the government the power to care for and protect persons who are legally declared unable to act on their own behalf. The doctrine of *police power* refers to the government's obligation to preserve societal order and protect the public from harm. Both powers are limited by the provisions of the US Constitution that protects civil rights of individuals, such as the right to privacy; the right to protection from the interference of the government in private matters, including medical decisions; the right to freedom of thought and expression; and the right to freedom from bodily restraints and confinement [19–21].

Many individuals with mental illness in early American communities were cared for by their family members, confined in county jails, or detained in shelters for the poor [2, 3, 22–26]. The first organized societal effort to care for the mentally ill in the US began in 1752, when Quakers opened Pennsylvania Hospital in Philadelphia. In the nineteenth century, social reformers such as Dorothea Dix fought for more humane alternative to jails and poorhouses. In response to their efforts, the first generation of American mental asylums were created, and the states began to assume increasing responsibility for the care of indigent mentally ill. Admissions of individuals with mental illness to asylums were often initiated at the request of family members who were willing to cover the cost of care or overseers of the poor representing indigent persons. The sole requirement for admission was that the individual needed treatment. Admission on an involuntary basis was an accepted norm since the mental illness was thought to compromise mental faculties to such an extent that patients would not be capable of requesting care or consenting to care [2, 3].

Although the establishment of early asylums was motivated by humanitarian impulses, patients in such institutions suffered from stigmatization, clinicians' limited understanding of neuropsychiatric disorders, and lack of effective treatments [19, 21–26]. In the absence of effective treatments, interventions consisted primarily of restraints, sedation with medications such as bromides and chloral hydrate, or exposure to various experimental treatments such as purging, bloodletting, cold or hot water immersions, and prayer [20–25]. Many of the chronically symptomatic

patients were subjected to trepanations (having holes drilled into the skull) or being placed in "tranquilizing" or "gyrating" chairs. Many patients remained confined with virtually no rights for the rest of their lives. As such, early asylums primarily served to sequester individuals with mental illness from the rest of society [2, 19, 21]. With time, asylums became notorious for their poor living conditions, lack of hygiene, overcrowding, and widespread abuse. In the mid-nineteenth century, several cases began to challenge the exiting norms.

The case of Josiah Oakes in 1845 first articulated the basis for civil commitment in the US [27]. Mr. Oakes was committed at McLean Asylum in Massachusetts at the behest by his children after he became engaged to a young woman within days of his wife's death. After 4 years, he challenged his commitment. Even though the Massachusetts Supreme Court decision was not favorable for Mr. Oakes, it began to establish criteria to be used for involuntary hospitalization. In 1860, Elizabeth Packard successfully challenged her 3-year-long confinement to the Illinois State Hospital at the behest of her husband who committed her after she challenged his religious beliefs [25]. At that time, Illinois statute allowed for the commitment of wives by their husbands without wives' consent or any formal hearing. Following Ms. Packard's release from the hospital, her lobbying efforts on behalf of disempowered women contributed to changing the commitment laws in four states [25]. These cases were instrumental in initiating societal debates on the substantive criteria and procedural mechanisms for involuntary commitment and resulted in significant legal reform by the end of the nineteenth century that established safeguards against unjust commitments.

In the first half of the twentieth century, commitment statutes based on parens patriae doctrine predominated, requiring only the presence of mental illness and the need for treatment. Statutes often equated involuntary hospitalization with global incompetency and those civilly committed suffered many violations of their basic human rights, including the right to register to vote [2, 3, 28]. In the 1950s, the US nationwide inpatient census peaked at around half a million, with many individuals remaining hospitalized for decades with little or no hope of relieving their symptoms, discharge from the hospital, or resuming a meaningful life outside of an institution. At that time, a confluence of cultural and political factors altered the practice of psychiatry and affected civil commitment laws in the United States [2–4, 6]. In 1946, in response to increasing number of mental health problems in Second World War veterans, President Truman signed the National Mental Health Act, calling for the establishment of the National Institute of Mental Health to advance treatment and understanding of psychiatric disorders. The discovery of the mood-stabilizing effect of lithium carbonate in 1948 and antipsychotic effect of chlorpromazine in 1955 dramatically advanced psychiatric treatment. In the 1960s, the civil rights movement exposed the abysmal conditions in state asylums. Together with criticism from within the psychiatric profession and increase in governmental funding for community support, these events shifted paternalistic commitment criteria to criteria based on dangerousness and grave disability [2–4, 6] and initiated the process of replacing long confinements in psychiatric hospitals with community mental health services. In 1964, Washington, DC, instituted a standard for civil commitment

establishing that prior to involuntary hospitalization a person must be found to have mental illness and pose a threat of harm to self or others, or be assessed gravely disabled [2, 3, 19]. Implementation of similar statutes in other states followed until it became the prevailing standard for civil commitment in the USA.

Several landmark cases from that period added additional substantive and procedural safeguards. In 1966, in the case of Lake v. Cameron, the US Court of Appeals for the District of Columbia Circuit ruled that civilly committed patients may not be held in a hospital if a less restrictive alternative for treatment delivery is available [29]. Similar reasoning was subsequently adopted by most states with only a few exceptions [2, 3, 19, 21, 29]. The most radical legal protection of patients' autonomy came from the Wisconsin Federal District Court, in the case of Lessard v. Schmidt in 1971 [30]. Ms. Alberta Lessard challenged mental health professionals' assessment that she should be permanently committed to a state psychiatric hospital as a result of her schizophrenia. She filed a class action lawsuit on behalf of all involuntarily committed adults in Wisconsin arguing that Wisconsin's involuntary commitment statute violated her due process rights. The court ruled in her favor, reasoning that because civil commitment represented a significant deprivation of liberty, the state was required to prove mental illness and dangerousness "beyond a reasonable doubt" and that a less restrictive treatment setting was unavailable [30]. The decision in Lessard established several rights due to the confined individual, including the right to a jury trial, the right to counsel, the right to exclude hearsay evidence, and the right to have it proven that the individual is both mentally ill and dangerous.

In 1975, the US Supreme Court heard the case of Kenneth Donaldson, who was committed to Florida State Hospital following a petition of his father [31]. After about 15 years of treatment with "milieu therapy," no significant psychotherapy or pharmacotherapy, and having his repeated requests for discharge denied, Mr. Donaldson sued the hospital contending that his constitutional right to liberty had been violated without providing adequate treatment. The court found Mr. Donaldson's commitment unconstitutional holding that, "A State cannot constitutionally confine a non-dangerous individual who is capable of surviving safely in freedom by themselves or with the help of willing and responsible family members or friends" [31]. In 1979, as discussed above, Addington v. Texas case addressed the issue of burden of proof in civil commitment hearings [1]. The Court in Addington balanced individual liberty interests against the societal interest in committing dangerous mentally ill individuals. The Court reasoned that possible injury to the individual from an erroneous decision is significantly greater than any possible harm to the state; therefore, the state must justify confinement by a standard higher than the "preponderance of the evidence." The court also distinguished civil commitment proceedings from criminal proceedings which require "beyond a reasonable doubt standard," emphasizing that civil commitment is not a punishment. Rather, individuals with mental illness are entitled to treatment, to periodic and recurrent review of their mental condition, and to release when they are no longer dangerous to themselves or others [1–4, 6]. Decisions in these historic legal cases reverberate in contemporary commitment statutes.

Practical Aspects of Contemporary Commitment Processes

Evaluations to assess whether to involuntarily commit patients are usually performed in emergency settings where patients are brought by the ambulance, police, family, friends, or coworkers. Common reasons that prompt such evaluations include acute mental status changes, expressions of suicidal or homicidal ideation, engagement in self-injurious or violent behavior, or the inability to care for oneself. Additionally, patients may be referred by their outpatient clinicians when assessed to be at risk for self-harm or harming others. If, after a thorough evaluation and review of collateral information, the patient is assessed to be in need of acute psychiatric hospitalization, the patient can be offered a voluntary admission. If the patient declines hospitalization or is deemed to not have the capacity to make such a decision, two mechanisms for involuntarily hospitalizing the patient are generally available to the psychiatrist—emergency commitment and civil commitment [3, 6, 19, 21].

Emergency Commitment

Relying on emergency commitment laws, psychiatrists may initiate an emergency commitment for which court involvement is often not initially required for a brief period, ranging from 3 days to 2 weeks. Most states require that a physician or psychologist sign the commitment certificate, but some states require more than one professional to sign, and some allow state agencies, such as the police or the courts, to initiate commitment when no mental health professional is available. The purpose of an emergency commitment is to ensure a patient's safety during a time of crisis and implementation of necessary mental health treatments [19, 21, 32].

In most states procedural safeguards are implemented to protect the rights of the patient to contest emergency confinement. For example, many states allow the patient to challenge their emergency confinement in probate court; in some states such hearings are referred to as a "probable cause hearings." At the hearing, the decision-maker, usually a probate court judge, is charged with determining whether a probable cause existed to believe that the patient met emergency commitment standards at the time of the initial psychiatric evaluation. At the conclusion of the hearing, the judge may decide to either release the patient or commit them to the hospital for the remaining duration of the emergency certificate [21, 32].

A 2016 review of the US emergency commitment laws by Hedman [32] found that even though every state in the United States and the District of Columbia has emergency commitment laws, states' laws varied regarding the duration allowed for emergency holds, who can initiate an emergency hold, the extent of judicial oversight, and the rights of patients during the hold. The core criterion justifying an involuntary hold in all states was the presence of mental illness that results in danger to self or others, but many states had additional specifications. Only 22 states required some form of judicial review of the emergency commitment process, and only nine required a judge to certify the commitment before a person is

hospitalized. Five states did not guarantee an assessment by a qualified mental health professional during the emergency hold [32].

Civil Commitment

Civil commitment is a more complex process than emergency commitment and involves filing a petition for civil commitment with the court of proper jurisdiction. This process is typically utilized in two types of situations. First, if a patient committed on an emergency basis is determined to still require hospitalization beyond the duration of the emergency commitment certificate, civil commitment procedures may be invoked prior to the expiration of the emergency commitment. Second, when a patient who was initially hospitalized voluntarily requests discharge and the clinical team determines that discharge would be unsafe, the team may petition the court for civil commitment in an attempt to continue treatment [2, 19, 21]. In some states, when an inpatient has been adequately stabilized but has a history of treatment noncompliance and frequent hospitalizations, an evaluation for outpatient civil commitment may be initiated prior to discharge [19, 21, 33–35].

Involuntary Outpatient Commitment

Following hospitalization, some psychiatric patients discontinue or have no access to treatment and their condition may rapidly decompensate. To address concerns related to frequent rehospitalizations and risk of dangerousness in the community as well as, increase access to needed care, many states have developed involuntary outpatient commitment or assisted outpatient treatment programs [33–38]. Such programs offer intensive outpatient services, including medication management and a variety of psychosocial services, and are usually coordinated by a case manager. Although treatment plans may include adherence to psychotropic medications (when indicated), involuntary medication administration on an outpatient basis is subject to the same review processes as when patients are hospitalized. As of 2015, 45 states and the District of Columbia have commitment statutes permitting involuntary outpatient commitment. States usually provide due process protections equivalent to those afforded patients subjected to involuntary hospitalization. Despite protections, several ethical concerns have been raised about the excessive use of involuntary outpatient commitment when there is insufficient evidence of the patient's risk of dangerousness, its disproportionate use for patients identifying as racial and/or ethnic minority groups, and the inability of states to implement the treatment programs mandated by these laws due to inadequate state resources [35–37].

Involuntary Commitment of Minors

The US Supreme Court granted due process rights to minors in 1967, in the case of In re Gault in 1967 [39]. The case involved an adolescent boy, Gerald Gault, who had been found guilty of making obscene phone calls to his neighbor and subsequently committed to a state industrial school until his adulthood without notification to his parents. Gault's parents challenged his commitment. The Court ruled that minors accused of crimes must be afforded many of the same due process rights as adults, such as the right to timely notification of the charges, the right to confront witnesses, the right against self-incrimination, and the right to counsel. A decade later, in 1979, the US Supreme Court established a constitutional minimum for involuntary psychiatric commitment of minors, holding that there was no requirement for an adversarial hearing. Rather, a "neutral fact finder" (e.g., a doctor not involved in the patient's care) would review and oversee cases, placing a greater emphasis on medical rather than judicial decision-making [40]. This decision was later challenged on the grounds that it violated due process by not providing adequate protections for mature minors and wards of the state [41].

Conclusion

In psychiatry, as in all of medicine, refusal of care by patients who need it most presents particular challenge. Current legal standards and procedural safeguards have been developed over the past two centuries in an attempt to balance safety concerns and patient liberty interests. These standards continue to evolve and may differ across states, patients age, and clinical contexts, including emergencies, inpatient, and outpatient settings. In order to successfully balance competing interests at stake in involuntary commitment while remaining considerate of inherent ethical concerns, mental health professionals should ensure familiarity with continuing evolution of commitment laws as well as legal guidelines in their particular area of practice.

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